

Mental Health — Depression, Anxiety, and Common Disorders

1. Overview and Burden in India

Mental health disorders are a leading cause of disability worldwide. The WHO estimates that 1 in 8 people globally lives with a mental disorder. In India, the National Mental Health Survey 2015-16 found a 13.7% prevalence of mental health disorders in adults, amounting to approximately 150 million people in need of care.

Despite this burden, India's mental health treatment gap is enormous — over 80% of people with mental disorders in India do not receive treatment. This gap is driven by stigma, lack of mental health professionals (approximately 0.3 psychiatrists per 100,000 population vs the recommended 3), economic barriers, and cultural factors.

The National Mental Health Programme (NMHP) and District Mental Health Programme (DMHP) are government initiatives to integrate mental health into primary care. India's Mental Healthcare Act 2017 guarantees the right to mental healthcare and prohibits discrimination against people with mental illness.

2. Depression (Major Depressive Disorder)

Major Depressive Disorder (MDD) is characterised by persistent low mood and loss of interest/pleasure (anhedonia) lasting at least 2 weeks, causing significant impairment. WHO ranks depression as the single largest contributor to global disability.

Diagnostic criteria (DSM-5, 5 or more in 2 weeks): depressed mood most of the day, markedly diminished interest/pleasure, significant weight loss or gain, insomnia or hypersomnia, psychomotor agitation or retardation, fatigue, feelings of worthlessness or excessive guilt, poor concentration/indecisiveness, recurrent thoughts of death or suicidal ideation.

Biopsychosocial model: depression arises from an interaction of biological factors (monoamine dysregulation, HPA axis hyperactivity, neuroinflammation, genetic vulnerability — heritability 40%), psychological factors (cognitive distortions, learned helplessness, negative attributional style), and social factors (adverse life events, poverty, trauma, isolation).

In India, presentation is often somatic (bodily complaints) rather than emotional — pain, fatigue, sleep disturbance — due to cultural somatisation and stigma against 'mental' complaints. This leads to frequent misdiagnosis and investigation without psychiatric evaluation.

3. Treatment of Depression

Antidepressants are the cornerstone of moderate-severe depression treatment. SSRIs (selective serotonin reuptake inhibitors) are first-line: fluoxetine 20 mg, sertraline 50-100 mg, escitalopram 10-20 mg. Onset of therapeutic effect: 4-6 weeks. Continue for minimum 6-9 months after remission (first episode).

SNRIs (venlafaxine, desvenlafaxine, duloxetine): add noradrenaline reuptake inhibition. Useful in depression with anxiety and pain. Duloxetine also indicated for diabetic neuropathy.

Mirtazapine: sedating antidepressant, useful when insomnia and anxiety are prominent. Promotes weight gain — can be beneficial in underweight patients.

Psychotherapy is equally effective as medication for mild-moderate depression and better for preventing relapse. Cognitive Behavioural Therapy (CBT) — most evidence-based. Interpersonal Therapy (IPT). Problem Solving Therapy (PST). Behavioural Activation.

Electroconvulsive Therapy (ECT): indicated for severe, treatment-resistant depression, psychotic depression, catatonia, and when rapid response needed (severe suicidal ideation, refusing food and water). Safe and effective; memory side effects are usually temporary.

4. Anxiety Disorders

Generalised Anxiety Disorder (GAD): excessive, uncontrollable worry about multiple everyday concerns lasting ≥ 6 months. Physical symptoms: muscle tension, restlessness, fatigue, poor concentration, irritability, sleep disturbance. Treatment: SSRIs/SNRIs, buspirone, CBT (cognitive restructuring, worry time, relaxation).

Panic Disorder: recurrent unexpected panic attacks (sudden surge of intense fear/discomfort reaching a peak within minutes — palpitations, sweating, trembling, shortness of breath, chest pain, dizziness, fear of dying or 'going crazy') + persistent concern about future attacks.

Social Anxiety Disorder (Social Phobia): intense fear of social situations due to fear of embarrassment or negative evaluation. Significant impairment. Treatment: CBT (exposure therapy), SSRIs, moclobemide.

Specific Phobias, OCD, PTSD are related anxiety-spectrum disorders with specific treatments.

5. Psychosis and Schizophrenia

Schizophrenia: a severe mental disorder characterised by positive symptoms (hallucinations — usually auditory; delusions — fixed false beliefs; disorganised thinking, bizarre behaviour), negative symptoms (flat affect, alogia, avolition, anhedonia, social withdrawal), and cognitive symptoms (working memory deficits, impaired executive function).

Prevalence: approximately 1% globally. Onset typically late teens to mid-30s. Course is highly variable: some achieve full remission, many have episodic course, some develop chronic disability. Early intervention (within the first episode) significantly improves long-term outcomes.

Antipsychotic medications: second-generation (atypical) antipsychotics preferred due to lower EPS risk: olanzapine, quetiapine, risperidone, aripiprazole, clozapine (for treatment-resistant schizophrenia). First-generation: haloperidol, chlorpromazine. Long-acting injectable (LAI) antipsychotics improve adherence for those with poor oral medication compliance.

6. Substance Use Disorders

Alcohol Use Disorder (AUD): India is the third largest consumer of alcohol globally. Alcohol is involved in 20-30% of hospital admissions in India. Alcohol dependence — physical and psychological dependence, tolerance, withdrawal (tremors, sweating, seizures — potentially fatal delirium tremens within 24-72 hours of last drink).

Treatment of alcohol withdrawal: benzodiazepines (diazepam or chlordiazepoxide — 'front-loading' CIWA-Ar guided protocol). IV thiamine before glucose (Wernicke's encephalopathy prevention). Long-term abstinence: naltrexone, acamprosate, disulfiram (causes unpleasant reaction if alcohol consumed), psychosocial support (Alcoholics Anonymous, SMART Recovery).

Opioid Use Disorder: in India, opioid dependence is significant in Punjab, North-Eastern states, and injecting drug user (IDU) populations. Opioid substitution therapy (OST): buprenorphine-naloxone (Suboxone) or methadone — reduces illicit drug use, HIV transmission, criminal activity, and mortality.

7. Child and Adolescent Mental Health

ADHD (Attention Deficit Hyperactivity Disorder): one of the most common neurodevelopmental disorders. Inattention, hyperactivity, impulsivity in ≥ 2 settings (school, home). Diagnosis requires symptoms before age 12, causing significant impairment. Treatment: methylphenidate (Ritalin), behavioural therapy, parent training, educational support.

Autism Spectrum Disorder (ASD): characterised by persistent deficits in social communication and interaction, and restricted/repetitive patterns of behaviour. Early identification and intensive behavioural intervention (ABA, speech therapy) significantly improve outcomes.

Depression and anxiety are common in adolescents in India — academic pressure, social media, bullying, and family stress are major risk factors. iCall and Vandrevalla Foundation helplines provide adolescent mental health support.

8. Suicide Prevention

India accounts for approximately 17% of global suicides. The age-standardised suicide rate in India is 12.0 per 100,000 (NCRB 2021 data). Suicide is the leading cause of death in the 15-29 age group in India. Common methods: pesticide ingestion (particularly in rural areas), hanging.

Risk factors: previous suicide attempt (strongest predictor — 30x increased risk), mental illness (depression, alcohol use disorder, schizophrenia), chronic pain/illness, social isolation, family history, access to means, hopelessness, recent loss or humiliation.

Protective factors: strong social connections, religious or cultural beliefs against suicide, children at home, reasons for living, access to mental health care, restriction of means (locking away medications and pesticides).

Key intervention: asking about suicidal ideation does NOT increase risk (Safe Messaging guidelines). Direct question: 'Are you having thoughts of suicide?' Assess plan, means, and intent. If at risk: do not leave alone, remove means, facilitate urgent psychiatric evaluation, involve family.

iCall Helpline (TISS): 9152987821. Vandrevalla Foundation: 1860-2662-345 (24/7). NIMHANS Helpline: 080-46110007. National Suicide Prevention Helpline: 9152987821.

9. Dementia

Dementia is a syndrome characterised by progressive decline in memory, language, problem-solving, and other cognitive domains sufficient to interfere with daily life. Over 55 million people live with dementia globally; 8-9 million in India.

Alzheimer's disease (60-70%): amyloid plaques and neurofibrillary tangles. Insidious onset, gradual progression. Cholinesterase inhibitors (donepezil 5-10 mg, rivastigmine, galantamine) — modest symptom benefit. Memantine (NMDA antagonist) for moderate-severe. Aducanumab and lecanemab (anti-amyloid antibodies) — first disease-modifying treatments showing modest benefit in trials.

Vascular dementia: second most common. Step-wise progression after strokes. Control vascular risk factors (BP, diabetes, cholesterol, smoking). No specific pharmacotherapy.

Carer burden: majority of dementia care in India falls on family, predominantly women. Caregiver stress, depression, and burnout are major concerns. Alzheimer's and Related Disorders Society of India (ARDSI) provides support resources.

10. Mental Health First Aid and Self-Care

Mental Health First Aid (MHFA): a structured programme training non-professionals to help someone experiencing mental health crisis. Key skills: approach the person, listen non-judgementally, give reassurance, encourage professional help, encourage self-help strategies.

Self-care practices for mental health: regular physical exercise (30 min/day reduces depression risk by 48%), sleep hygiene, mindfulness meditation (6-8 weeks of MBSR reduces anxiety and depression symptoms), social connection, limiting alcohol, nature exposure.

For students and professionals: recognise burnout (emotional exhaustion, depersonalisation, reduced sense of accomplishment). Strategies: set boundaries on work hours, maintain hobbies and social relationships, seek mentorship, use employee assistance programmes (EAP).

Reducing stigma: use person-first language ('person with schizophrenia' not 'schizophrenic'). Share recovery stories. Challenge mental health misinformation. In India, initiatives like iDare, Mpower, Lissun, and Vandrevalla Foundation are working on digital mental health and awareness.